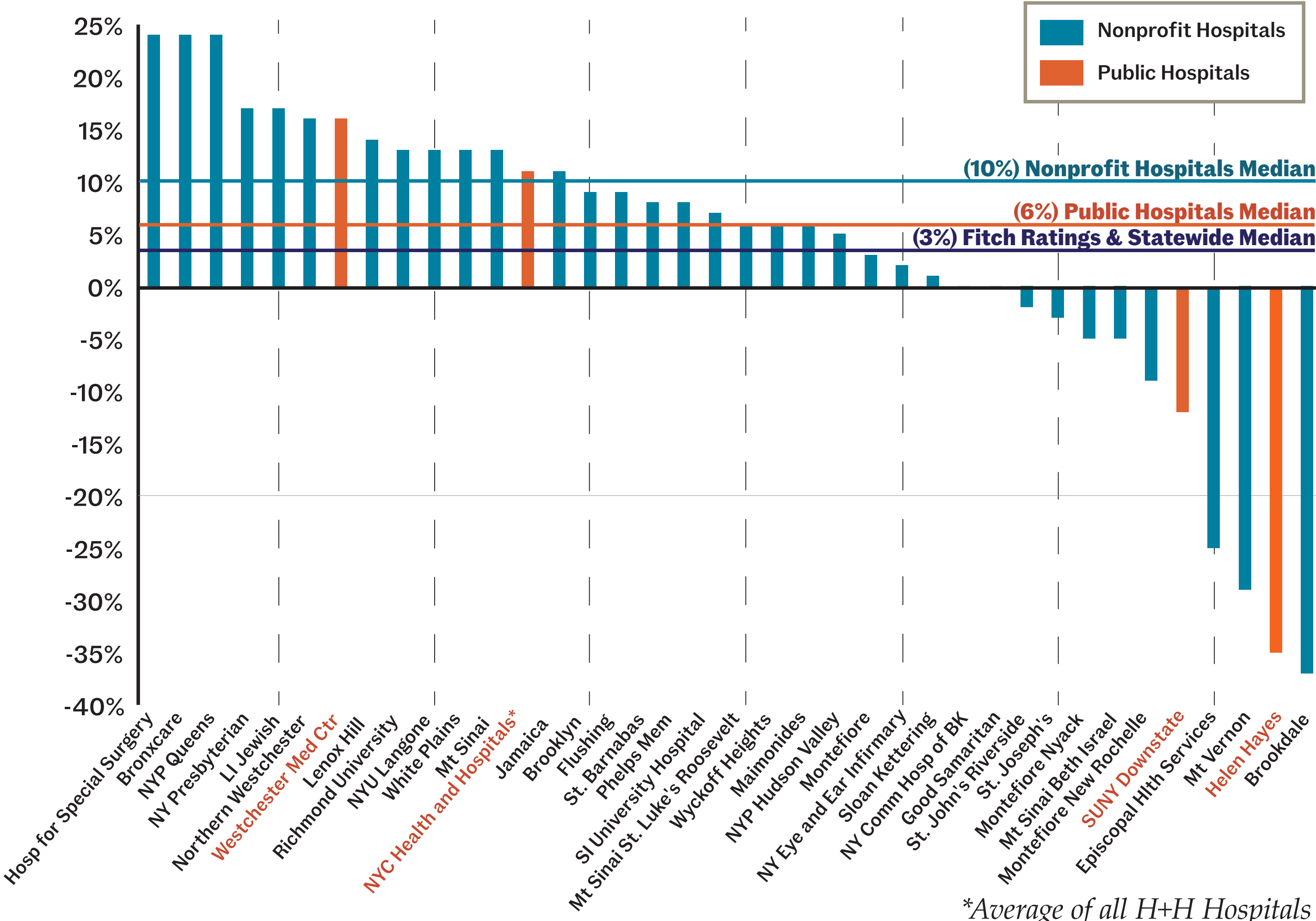


NEW YORK CITY HOSPITALS

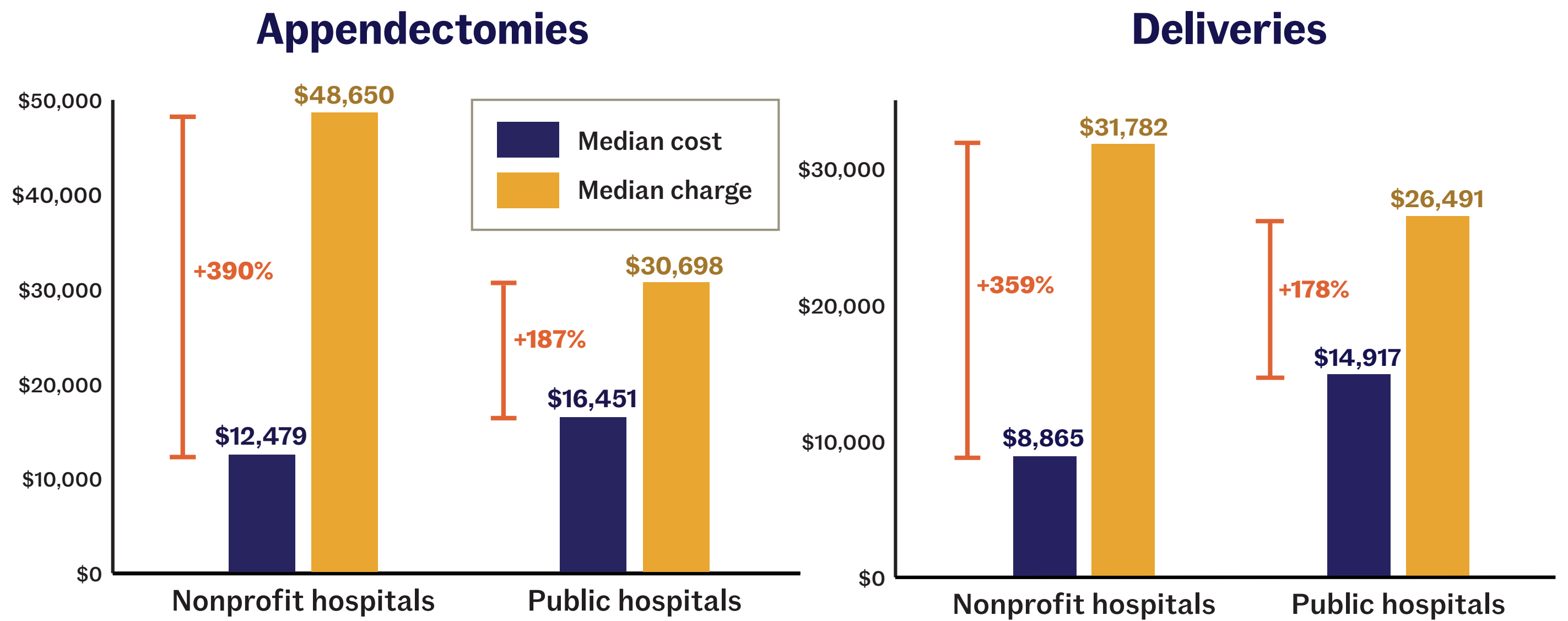
Hospital operating profit margins in the New York City Metro area



*Average of all H+H Hospitals

Private nonprofit hospitals (34) had a median operating profit of over 10% in 2022, compared to 6% for public hospitals (13). Both have operating profit margins well above the Statewide median and the 3% Fitch Ratings considered healthy for nonprofit hospitals to meet their obligation.

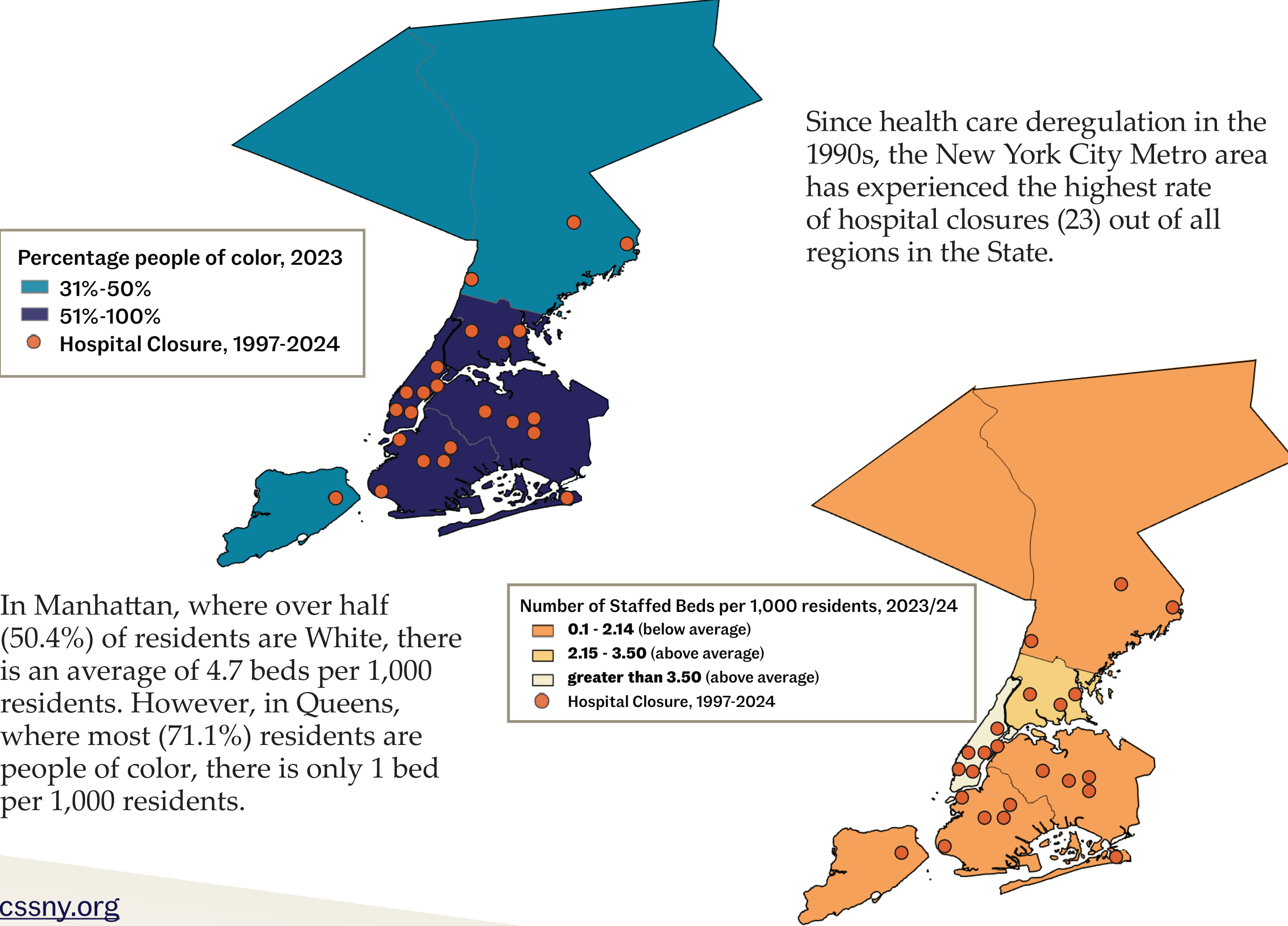
Median costs and charges by procedure in the New York City Metro area



In the New York City Metro area, nonprofit hospitals charge almost **four times** their costs for appendectomies, while public hospitals charge nearly twice their costs. For vaginal deliveries, nonprofit hospitals charge **3.5 times** their costs, while public hospitals charge only 1.75 times their costs.

Over a third of the New York City Metro area's hospitals (19 of 47) belong to the State's six largest health systems. Consolidation in the New York City Metro market has let its hospitals, especially private nonprofit hospitals, leverage their market power to command high charges for common procedures.

Disparities in access to care in the New York City Metro area

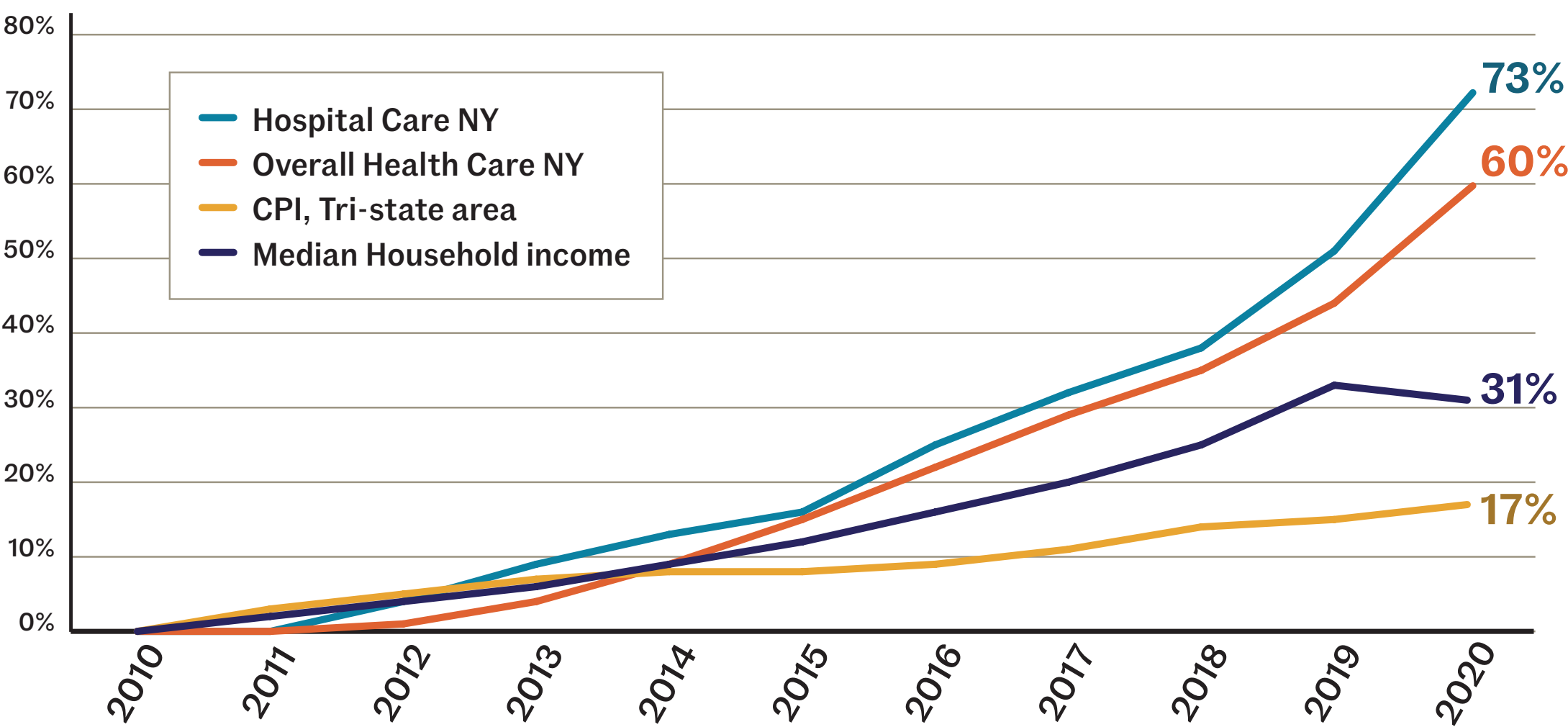


In Manhattan, where over half (50.4%) of residents are White, there is an average of 4.7 beds per 1,000 residents. However, in Queens, where most (71.1%) residents are people of color, there is only 1 bed per 1,000 residents.

Why is Health Care So Unaffordable In New York?

High hospital prices adversely affect payers, employers, unions, and patients alike, resulting in high insurance costs, elevated patient cost-sharing, and medical debt.

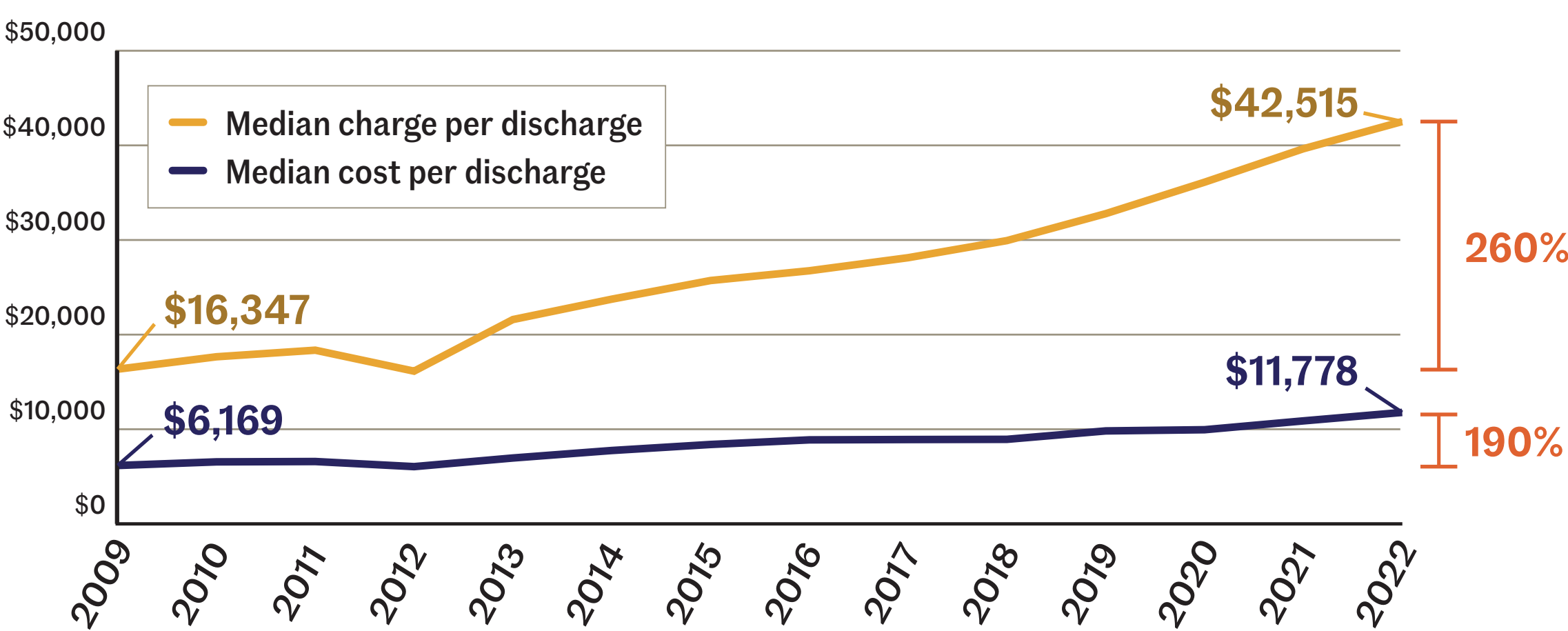
Rise in health care expenditures outpaces income and inflation, 2010-2020



New York’s health care spending has tripled over the past three decades. In 2020, its per capita health care spending was the second-highest in the nation at \$14,000. Since 2010, spending on hospital care rose **twice** as fast as income and **four times** as fast as inflation.

Hospital charges outgrow reported-costs for inpatient appendectomies, 2009-2022

Hospital charges appear to be a significant driver of the rapid increase in New York’s health care spending. The amount hospitals charge **rose substantially faster** than the reported cost of providing that same care (260% compared to 190%).



New York hospital markets have become more highly concentrated, 2022

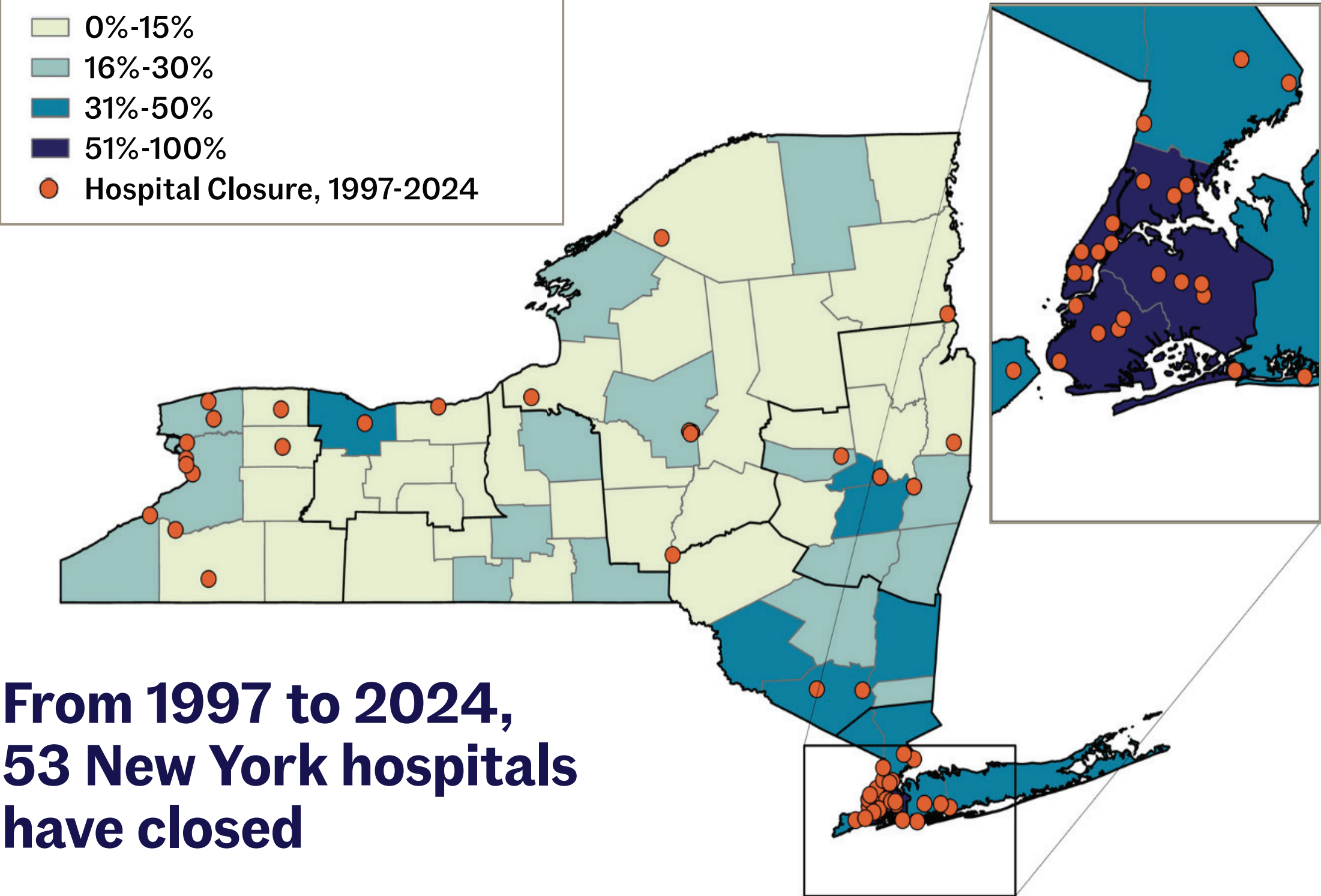
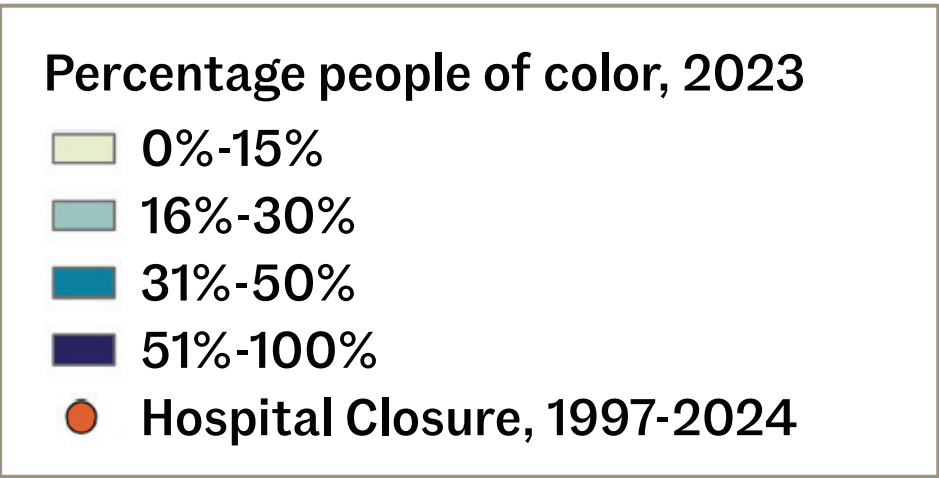
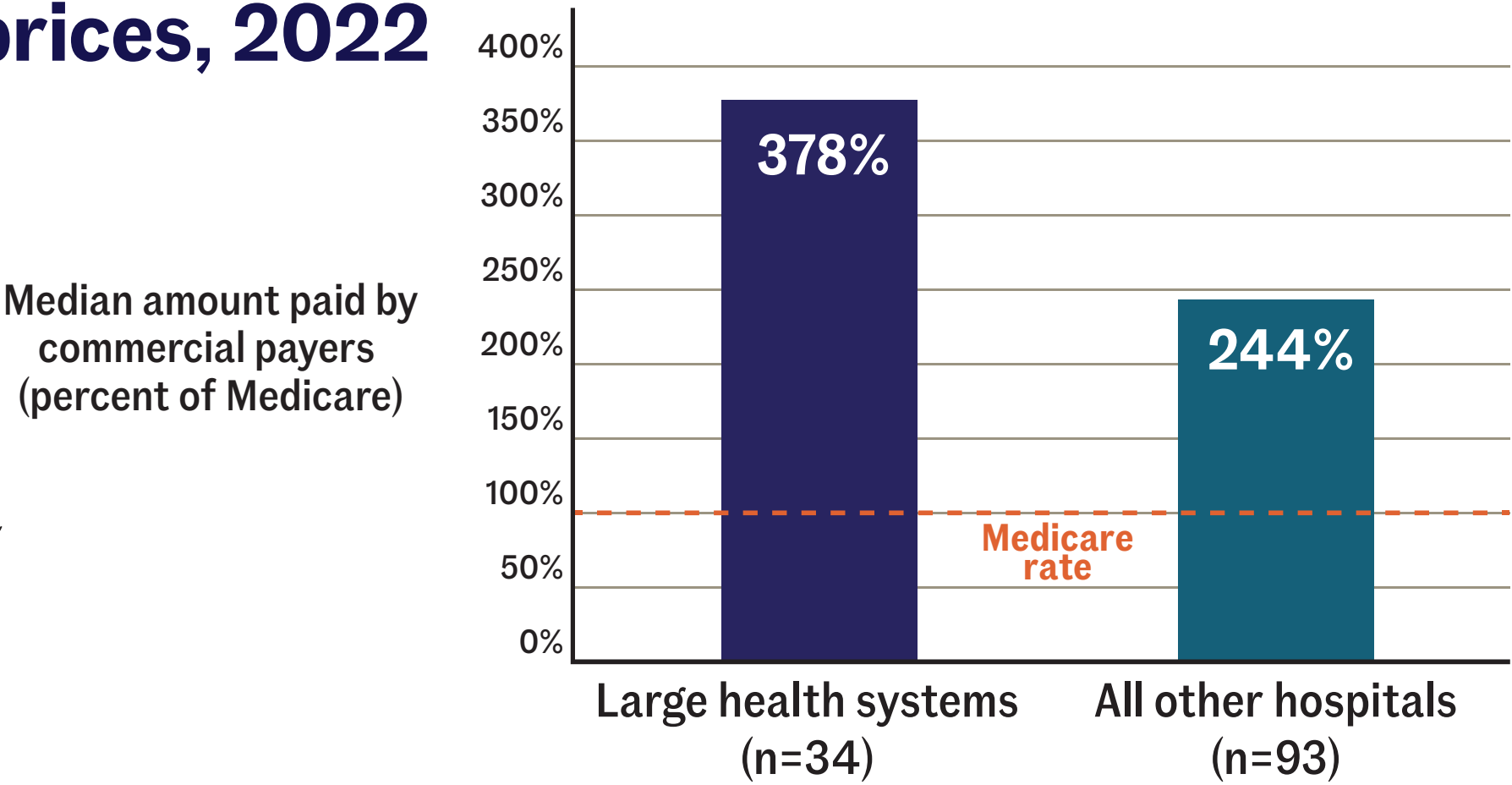
HIGHLY CONCENTRATED (HHI>1800)	NOT HIGHLY CONCENTRATED (HHI ≤1800)
Finger Lakes	Central and Southern Tier
Long Island	North Country and Mohawk Valley
Mid-Hudson	NYC Metro
Capital Region	
Western New York	

Consolidation has resulted in five out of eight New York regions meeting the Federal Trade Commission threshold for a “highly concentrated” hospital market. The NYC Metro area is still affected by consolidation, as half of the private hospitals in the city belong to the State’s largest private health systems.

New York’s six largest private hospital systems charge high commercial prices, 2022

New York’s six largest private hospital systems* control over 42% of inpatient beds and charge a median commercial price that is almost **four times** the Medicare rate.

**Northwell Health, New York-Presbyterian, Mount Sinai, Montefiore Einstein, Catholic Health, and NYU Langone Health*



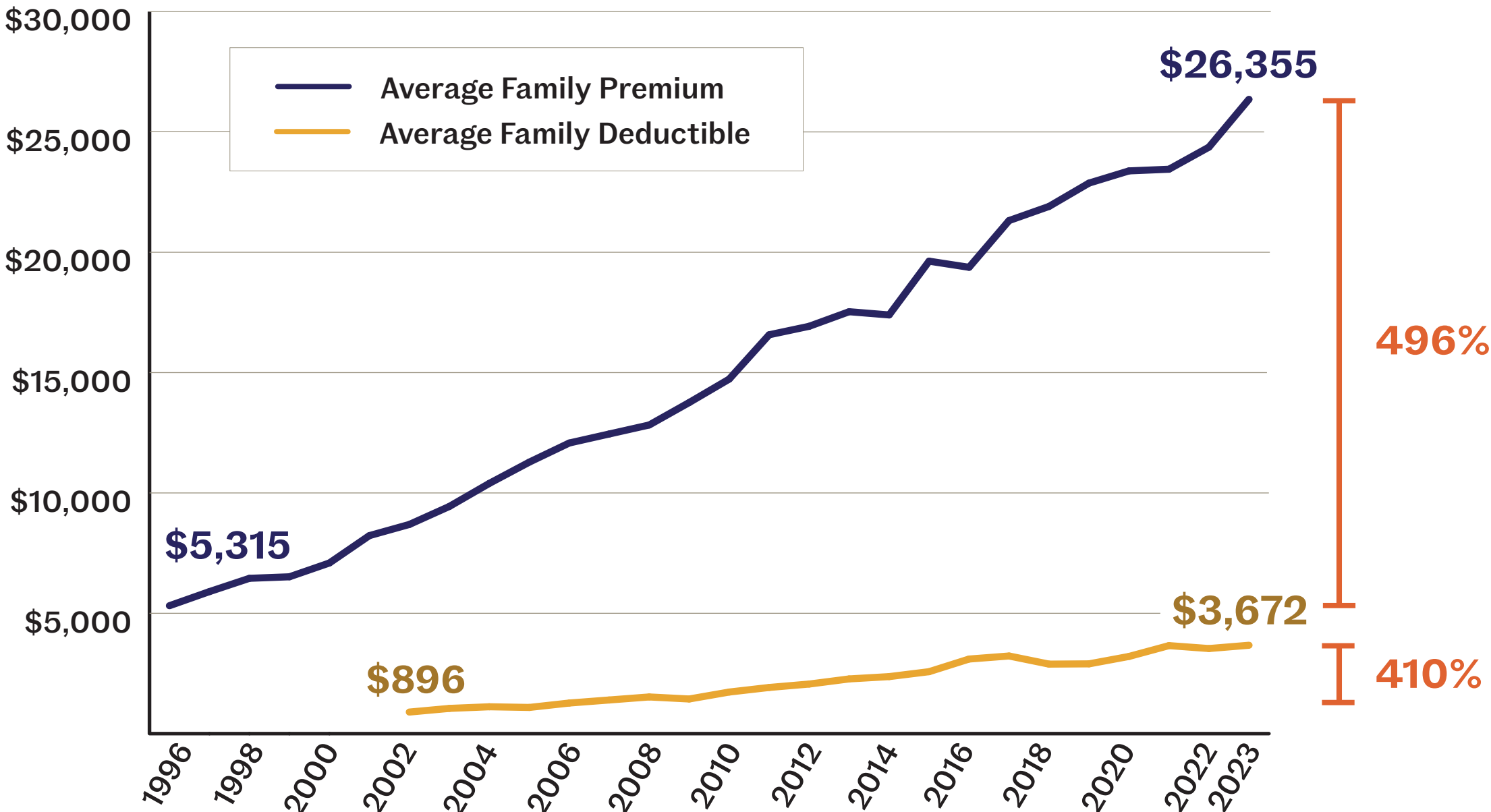
From 1997 to 2024, 53 New York hospitals have closed

As the State’s large health systems have grown, small hospitals have suffered. Hospital closures have not been equitably distributed across New York’s population, which has contributed to disparities in access to care. The median share of residents of color in the closure counties is 21% compared to just 12% in the non-closure counties.

Increase in average family premiums and deductibles, 1996-2023

Elevated hospital prices, resulting in substantial increases in insurance premiums, deductibles, and cost-sharing for patients, also lead to a higher medical debt burden.

New York has the second-highest health insurance premiums for family coverage in the nation.



New York hospitals rate poorly on quality of care, 2024

Overall quality rating	1 star	2 stars	3 stars	4 stars	5 stars
Number of hospitals	32	40	30	14	10

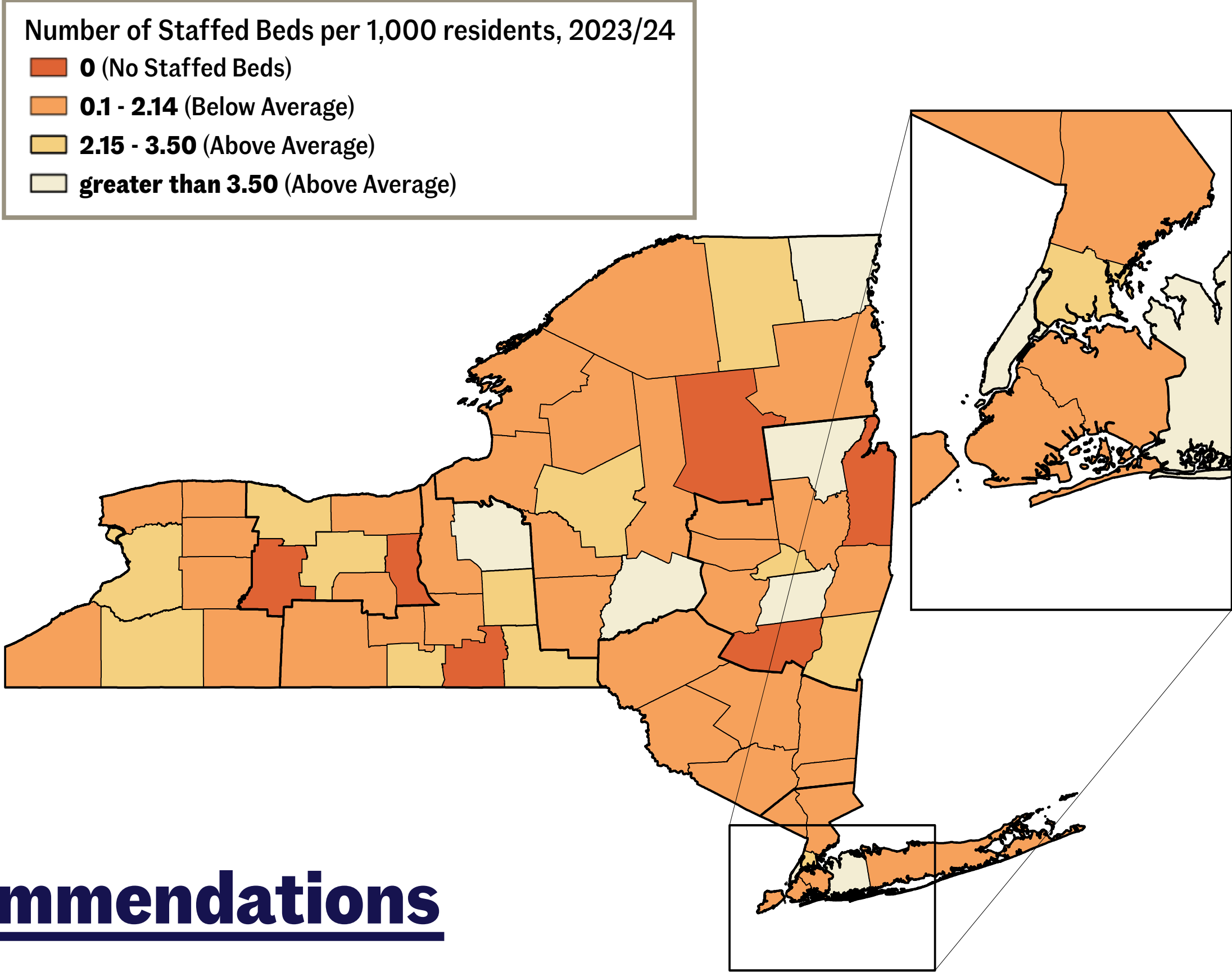
Only 19% of New York hospitals received 4- or 5-star CMS quality ratings. Nationwide, 40% of hospitals have a 4- or 5-star rating. The State’s hospitals rate poorly on multiple aspects of quality of care like: mortality; safety of care; readmissions; timely and effective care; and patient experience.

Access to hospital care varies widely across the State, 2023-2024

Over the past two decades, over 9,600 beds have been removed due to closures, consolidation, and other hospital restructuring (19% of the State’s total beds).

Six counties lack any staffed hospital beds (Greene, Hamilton, Livingston, Seneca, Tioga, Washington).

The Statewide average is 2.14 staffed beds per 1,000 residents.



Policy Recommendations

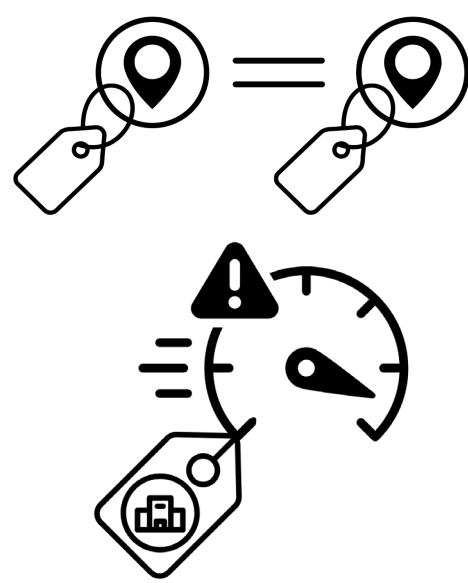
1. Adopt system-wide reforms.



The State should establish an independent entity like California’s Office of Health Care Affordability (OHCA) to: assess market consolidation; set limits on health care spending growth; and promote high-value care. An independent New York Office of Health Care Affordability could help address consolidation and inequitable hospital closures, slow health care spending growth, and address long-term structural issues that have reduced access to care, especially for New Yorkers who are low-income or people of color.

After 14 years, New York still has no functional, public-facing All Payer Claims Database (APCD). The State should implement the long-delayed APCD and follow the lead of at least eight states that use it to inform health care cost growth benchmarks and the 11 states that permit consumer price comparisons.

2. Impose targeted pricing reforms.



In the absence of a system-wide entity to redress New York’s health care affordability crisis, the State could adopt price caps for procedures at hospital outpatient departments by enacting the **Fair Pricing Act (S705|A2140)**. This law would ensure that the same price is charged for the same routine service across different sites of care, saving New Yorkers \$1.14 billion (\$213 million for consumers) annually.

The State should explore a hospital-specific cost review entity, such as the Diamond State Hospital Cost Review Board in Delaware, which sets limits on cost growth specifically for hospitals.

3. Establish a primary care spending target.



Seventy percent of emergency room visits in New York, over double the national rate, are non-emergent and could be better treated in a primary care setting. The **Primary Care Investment Act (S1634|A1915A)**, which passed in the Senate, would create a primary care spending benchmark to measure and report the percentage of carriers’ overall health care spending that goes towards primary care. Oregon, Colorado, Rhode Island, and Delaware have passed similar laws to increase primary care investment.